INDIVIDUAL CASE SAFETY REPORT

Overall case inventory

Case ID	Product	Reported event	MedDRA PT (provisional)	SOC (provisional)	Seriousness
SYN-PV-001	Amoxicillin	Rash	Rash	Skin and subcutaneous tissue disorders	Medically important
SYN-PV-002	Atorvastatin	Myalgia	Myalgia	Musculoskeletal and connective tissue disorders	Not serious
SYN-PV-003	Metformin	Nausea	Nausea	Gastrointestinal disorders	Hospitalisation
SYN-PV-004	Amlodipine	Peripheral edema	Edema peripheral	General disorders and administration site conditions	Medically important
SYN-PV-005	Ciprofloxacin	Tendon pain	Tendon pain	Musculoskeletal and connective tissue disorders	Medically important
SYN-PV-006	Sertraline	Dizziness	Dizziness	Nervous system disorders	Medically important
SYN-PV-007	Omeprazole	Headache	Headache	Nervous system disorders	Medically important
SYN-PV-008	Losartan	Fatigue	Fatigue	General disorders and administration site conditions	Not serious
SYN-PV-009	Amoxicillin	Rash	Rash	Skin and subcutaneous tissue disorders	Medically important
SYN-PV-010	Atorvastatin	Myalgia	Myalgia	Musculoskeletal and connective tissue disorders	Hospitalisation
SYN-PV-011	Metformin	Nausea	Nausea	Gastrointestinal disorders	Medically important
SYN-PV-012	Amlodipine	Peripheral edema	Edema peripheral	General disorders and administration site conditions	Medically important

Coding note: PT/SOC assignments are provisional working mappings based on the reported verbatim event and standard MedDRA terminology knowledge. The supplied pack does not state a MedDRA version or provide a licensed MedDRA browser export. Confirm exact LLT-to-PT selection, currency, and primary SOC against the organization's licensed MedDRA version before any production use.

SYN-PV-001 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	30 years
Sex	Male
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Rash
MedDRA PT	Rash (provisional)
SOC	Skin and subcutaneous tissue disorders (provisional)
Onset	17 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — source classifies the event as medically important. The specific medically important criterion/rationale is not supplied.
Outcome	Not recovered at follow-up
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Amoxicillin
Dose / regimen	20 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Anxiety (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Negative
Rechallenge	Positive
Concomitant medicines	None reported
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Consumer
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 30-year-old patient (Male) received Amoxicillin (20 mg once daily) by the oral route for Anxiety. Rash was reported 17 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Rash'. The reporter stated that treatment was stopped. Dechallenge was recorded as negative and rechallenge as positive. The reported outcome at follow-up was not recovered at follow-up. Concomitant medication(s): None reported. Reporter type: Consumer.

Case-level data quality / follow-up points

• Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details where available.
 • Document the specific medically important medical event rationale and clinical details supporting seriousness.
 • Clarify the rechallenge circumstances, timing, dose, recurrence, and clinical evidence; do not infer causality from the label alone.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-002 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	36 years
Sex	Unknown / not reported
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Myalgia
MedDRA PT	Myalgia (provisional)
SOC	Musculoskeletal and connective tissue disorders (provisional)
Onset	1 day(s) after treatment began; calendar onset date unknown.
Seriousness	Non-serious — source explicitly classifies the case as not serious.
Outcome	Recovered
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Atorvastatin
Dose / regimen	5 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Anxiety (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Unknown
Rechallenge	Negative
Concomitant medicines	None reported
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Pharmacist
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 36-year-old patient (sex not reported) received Atorvastatin (5 mg once daily) by the oral route for Anxiety. Myalgia was reported 1 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Myalgia'. The reporter stated that treatment was stopped. Dechallenge was recorded as unknown and rechallenge as negative. The reported outcome at follow-up was recovered. Concomitant medication(s): None reported. Reporter type: Pharmacist.

Case-level data quality / follow-up points

• Confirm patient sex if available.
 • Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Clarify unavailable dechallenge/rechallenge details if obtainable.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-003 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	34 years
Sex	Female
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Nausea
MedDRA PT	Nausea (provisional)
SOC	Gastrointestinal disorders (provisional)
Onset	13 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — hospitalization reported. The source does not provide admission date, duration, diagnosis, or discharge details.
Outcome	Recovered
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Metformin
Dose / regimen	5 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Anxiety (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Negative
Rechallenge	Negative
Concomitant medicines	Vitamin D
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Consumer
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 34-year-old patient (Female) received Metformin (5 mg once daily) by the oral route for Anxiety. Nausea was reported 13 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Nausea'. The reporter stated that treatment was stopped. Dechallenge was recorded as negative and rechallenge as negative. The reported outcome at follow-up was recovered. Concomitant medication(s): Vitamin D. Reporter type: Consumer.

Case-level data quality / follow-up points

• Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Clarify hospitalization criterion, admission/discharge dates, reason for admission, and whether the event caused or prolonged admission.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-004 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	61 years
Sex	Male
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Peripheral edemas
MedDRA PT	Edema peripheral (provisional)
SOC	General disorders and administration site conditions (provisional)
Onset	21 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — source classifies the event as medically important. The specific medically important criterion/rationale is not supplied.
Outcome	Recovered
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Amlodipine
Dose / regimen	500 mg twice daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Anxiety (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Positive
Rechallenge	Negative
Concomitant medicines	None reported
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Pharmacist
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 61-year-old patient (Male) received Amlodipine (500 mg twice daily) by the oral route for Anxiety. Peripheral edema was reported 21 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Edema peripheral'. The reporter stated that treatment was stopped. Dechallenge was recorded as positive and rechallenge as negative. The reported outcome at follow-up was recovered. Concomitant medication(s): None reported. Reporter type: Pharmacist.

Case-level data quality / follow-up points

• Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details where available.
 • Document the specific medically important medical event rationale and clinical details supporting seriousness.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-005 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	33 years
Sex	Unknown / not reported
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Tendon pain
MedDRA PT	Tendon pain (provisional)
SOC	Musculoskeletal and connective tissue disorders (provisional)
Onset	15 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — source classifies the event as medically important. The specific medically important criterion/rationale is not supplied.
Outcome	Not recovered at follow-up
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Ciprofloxacin
Dose / regimen	500 mg twice daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Infection (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Unknown
Rechallenge	Not done
Concomitant medicines	None reported
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Pharmacist
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 33-year-old patient (sex not reported) received Ciprofloxacin (500 mg twice daily) by the oral route for Infection. Tendon pain was reported 15 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Tendon pain'. The reporter stated that treatment was stopped. Dechallenge was recorded as unknown and rechallenge as not done. The reported outcome at follow-up was not recovered at follow-up. Concomitant medication(s): None reported. Reporter type: Pharmacist.

Case-level data quality / follow-up points

• Confirm patient sex if available.
 • Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Document the specific medically important medical event rationale and clinical details supporting seriousness.
 • Clarify unavailable dechallenge/rechallenge details if obtainable.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-006 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	31 years
Sex	Unknown / not reported
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Dizziness
MedDRA PT	Dizziness (provisional)
SOC	Nervous system disorders (provisional)
Onset	3 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — source classifies the event as medically important. The specific medically important criterion/rationale is not supplied.
Outcome	Recovering
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Sertraline
Dose / regimen	20 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Hypertension (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Positive
Rechallenge	Not done
Concomitant medicines	Paracetamol
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Physician
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 31-year-old patient (sex not reported) received Sertraline (20 mg once daily) by the oral route for Hypertension. Dizziness was reported 3 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Dizziness'. The reporter stated that treatment was stopped. Dechallenge was recorded as positive and rechallenge as not done. The reported outcome at follow-up was recovering. Concomitant medication(s): Paracetamol. Reporter type: Physician.

Case-level data quality / follow-up points

• Confirm patient sex if available.
 • Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Document the specific medically important medical event rationale and clinical details supporting seriousness.
 • Clarify unavailable dechallenge/rechallenge details if obtainable.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-007 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	63 years
Sex	Male
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Headache
MedDRA PT	Headache (provisional)
SOC	Nervous system disorders (provisional)
Onset	16 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — source classifies the event as medically important. The specific medically important criterion/rationale is not supplied.
Outcome	Recovered
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Omeprazole
Dose / regimen	500 mg twice daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Hypertension (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Negative
Rechallenge	Negative
Concomitant medicines	Paracetamol
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Consumer
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 63-year-old patient (Male) received Omeprazole (500 mg twice daily) by the oral route for Hypertension. Headache was reported 16 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Headache'. The reporter stated that treatment was stopped. Dechallenge was recorded as negative and rechallenge as negative. The reported outcome at follow-up was recovered. Concomitant medication(s): Paracetamol. Reporter type: Consumer.

Case-level data quality / follow-up points

• Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details where available.
 • Document the specific medically important medical event rationale and clinical details supporting seriousness.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-008 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	36 years
Sex	Male
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Fatigue
MedDRA PT	Fatigue (provisional)
SOC	General disorders and administration site conditions (provisional)
Onset	2 day(s) after treatment began; calendar onset date unknown.
Seriousness	Non-serious — source explicitly classifies the case as not serious.
Outcome	Recovering
Expectedness	Unexpected per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Losartan
Dose / regimen	5 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Infection (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Negative
Rechallenge	Not done
Concomitant medicines	Paracetamol
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Pharmacist
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 36-year-old patient (Male) received Losartan (5 mg once daily) by the oral route for Infection. Fatigue was reported 2 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Fatigue'. The reporter stated that treatment was stopped. Dechallenge was recorded as negative and rechallenge as not done. The reported outcome at follow-up was recovering. Concomitant medication(s): Paracetamol. Reporter type: Pharmacist.

Case-level data quality / follow-up points

• Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Clarify unavailable dechallenge/rechallenge details if obtainable.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-009 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	78 years
Sex	Unknown / not reported
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Rash
MedDRA PT	Rash (provisional)
SOC	Skin and subcutaneous tissue disorders (provisional)
Onset	7 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — source classifies the event as medically important. The specific medically important criterion/rationale is not supplied.
Outcome	Recovering
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Amoxicillin
Dose / regimen	5 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Anxiety (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Positive
Rechallenge	Not done
Concomitant medicines	Vitamin D
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Physician
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 78-year-old patient (sex not reported) received Amoxicillin (5 mg once daily) by the oral route for Anxiety. Rash was reported 7 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Rash'. The reporter stated that treatment was stopped. Dechallenge was recorded as positive and rechallenge as not done. The reported outcome at follow-up was recovering. Concomitant medication(s): Vitamin D. Reporter type: Physician.

Case-level data quality / follow-up points

- Confirm patient sex if available.
 • Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Document the specific medically important medical event rationale and clinical details supporting seriousness.
 • Clarify unavailable dechallenge/rechallenge details if obtainable.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-010 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	57 years
Sex	Female
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Myalgia
MedDRA PT	Myalgia (provisional)
SOC	Musculoskeletal and connective tissue disorders (provisional)
Onset	21 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — hospitalization reported. The source does not provide admission date, duration, diagnosis, or discharge details.
Outcome	Recovered
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Atorvastatin
Dose / regimen	5 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Infection (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Unknown
Rechallenge	Negative
Concomitant medicines	Paracetamol
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Pharmacist
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 57-year-old patient (Female) received Atorvastatin (5 mg once daily) by the oral route for Infection. Myalgia was reported 21 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Myalgia'. The reporter stated that treatment was stopped. Dechallenge was recorded as unknown and rechallenge as negative. The reported outcome at follow-up was recovered. Concomitant medication(s): Paracetamol. Reporter type: Pharmacist.

Case-level data quality / follow-up points

• Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Clarify hospitalization criterion, admission/discharge dates, reason for admission, and whether the event caused or prolonged admission.
 • Clarify unavailable dechallenge/rechallenge details if obtainable.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

SYN-PV-011 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	23 years
Sex	Unknown / not reported
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Nausea
MedDRA PT	Nausea (provisional)
SOC	Gastrointestinal disorders (provisional)
Onset	19 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — source classifies the event as medically important. The specific medically important criterion/rationale is not supplied.
Outcome	Recovering
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Metformin
Dose / regimen	5 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Hypertension (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Unknown
Rechallenge	Positive
Concomitant medicines	None reported
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Consumer
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 23-year-old patient (sex not reported) received Metformin (5 mg once daily) by the oral route for Hypertension. Nausea was reported 19 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'Nausea'. The reporter stated that treatment was stopped. Dechallenge was recorded as unknown and rechallenge as positive. The reported outcome at
follow-up was recovering. Concomitant medication(s): None reported. Reporter type: Consumer.

Case-level data quality / follow-up points

- Confirm patient sex if available.
 • Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Document the specific medically important medical event rationale and clinical details supporting seriousness.
 • Clarify the rechallenge circumstances, timing, dose, recurrence, and clinical evidence; do not infer causality from the label alone.
 • Clarify unavailable dechallenge/rechallenge details if obtainable.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product

SYN-PV-011 — ICSR narrative

1. Patient section
reference documents.

SYN-PV-012 — ICSR narrative

1. Patient section

Field	Recorded information / status
Patient identifier	Not provided; patient initials or other unique identifier are unknown.
Age	24 years
Sex	Female
Relevant medical history	Not supplied; unknown.

2. Reaction section

Field	Recorded information / status
Verbatim event	Peripheral edema
MedDRA PT	edema peripheral (provisional)
SOC	General disorders and administration site conditions (provisional)
Onset	4 day(s) after treatment began; calendar onset date unknown.
Seriousness	Serious — source classifies the event as medically important. The specific medically important criterion/rationale is not supplied.
Outcome	Not recovered at follow-up
Expectedness	Known per synthetic coding reference.

3. Drug section

Field	Recorded information / status
Suspect product	Amlodipine
Dose / regimen	5 mg once daily (recorded verbatim; not clinically validated)
Route	Oral
Indication	Hypertension (recorded verbatim)
Action taken	Treatment stopped, per narrative.
Dechallenge	Unknown
Rechallenge	Positive
Concomitant medicines	None reported
Treatment start/stop dates	Calendar dates not supplied; unknown.

4. Reporter section

Field	Recorded information / status
Reporter qualification	Pharmacist
Reporter name / contact / address / country	Not supplied; unknown.
Report receipt date / source reference	Not supplied; unknown.

Consolidated ICSR narrative

A 24-year-old patient (Female) received Amlodipine (5 mg once daily) by the oral route for Hypertension. Peripheral edema was reported 4 day(s) after treatment began. The reported reaction is coded provisionally to MedDRA PT 'edema peripheral'. The reporter stated that treatment was stopped. Dechallenge was recorded as unknown and rechallenge as positive. The reported outcome at follow-up was not recovered at follow-up. Concomitant medication(s): None reported. Reporter type: Pharmacist.

Case-level data quality / follow-up points

• Obtain patient identifier/initials, relevant medical history, exact treatment and event dates, and reporter contact details were available.
 • Document the specific medically important medical event rationale and clinical details supporting seriousness.
 • Clarify the rechallenge circumstances, timing, dose, recurrence, and clinical evidence; do not infer causality from the label alone.
 • Clarify unavailable dechallenge/rechallenge details if obtainable.
 • Verify dose and frequency against the original source; retain source value until confirmed.
 • Verify event coding and listedness against the current licensed MedDRA and product reference documents.

Cross-case quality review and completion checklist

The source dataset contains 12 synthetic cases. All cases contain an age, suspect product, dose, route, indication, event, relative onset interval, seriousness label; dechallenge/rechallenge status, concomitant medicine field, and reporter type. Several core ICSR elements are absent and must remain explicitly unknown rather than being invented.

ICSR element	Assessment
Identifiable patient	Age is supplied, but initials/unique patient identifier are not. Confirm whether the source's patient is sufficiently identifiable under the applicable reporting rules.
Identifiable reporter	Reporter qualification is supplied; reporter name and contact details are absent. Confirm whether the reporter is sufficiently identifiable.
Suspect product	Product name supplied in all cases; verify product identity, strength, batch, manufacturer, and exact regimen where available.
Adverse event	Verbatim event supplied in all cases; provisional PT/SOC mapping provided.
Chronology	Relative onset in days supplied; calendar dates and treatment start/stop dates absent.
Seriousness	Source label retained. For medically important cases, specific criterion is not described. Hospitalisation labels lack admission details.
Outcome	Recovered / recovering / not recovered derived from source narrative.
Medical history	Not supplied; unknown.
Expectedness	Synthetic reference status retained; not a substitute for approved local product information or reference safety information.

Audit trail :< /b> Source case IDs are preserved exactly. Source values are transcribed without clinical normalization except for provisional MedDRA coding and the plain-language outcome label. No causal relationship, diagnosis, dose appropriateness, or missing patient/reporter detail has been inferred.

Regulatory note :< /b> this pack does not specify a jurisdiction or submission framework. No regulatory submission timeline is assigned in this narrative report. Seriousness classifications reproduce the supplied source labels; reporting obligations and timelines must be determined under the applicable local regulations and company procedures.

Prepared as :< /b> Educational ICSR narrative compilation. Version : 1.0. Preparation date : 04 November 2026.